

Simulation Scenario Report: B20 Inpatient Operational Readiness – External Transfer for Pneumonia

1. Section I: Scenario Demographics

Identify Scenario Title: B20 Inpatient Operational Readiness – External Transfer for Pneumonia (IV Abx and O₂ Therapy) **Target Learning Group:**

- Duty Managers & Registration Staff
- ER Nursing Staff & ER Physicians
- Inpatient Nursing Staff (Building 20)
- Inpatient Physicians (On-call/Admitting)
- EMS/Porter Teams
- Laboratory & Pharmacy Staff

2. Section II: Scenario Developers Stakeholders:

- B20 Hospital Operations
- IT/Health IT
- Emergency and Inpatient Nursing Leadership

3. Section III: Curriculum Integration

Educational Goal: To validate the operational readiness, workflow efficiency, and system interoperability for an external patient transfer arriving at the ER for admission to the B20 Inpatient unit. **CRM Objectives:**

- **Inter-departmental Communication:** Demonstrate effective verbal and digital communication between the Duty Manager, ER, and Inpatient units.
- **Seamless Patient Transitions:** Ensure efficient patient hand-offs between EMS, ER, and the designated inpatient floor. **Operational Objectives:**
- **Registration Integrity:** Validate 100% accuracy in the transition from pre-arrival encounters to inpatient encounter registration.
- **Unit & Equipment Readiness:** Confirm the inpatient room is fully equipped with functional oxygen, suction, and IV pumps prior to patient arrival.
- **Clinical Interoperability:** Verify successful order entry and readiness for Laboratory (CBC) and Pharmacy (Imodium) services.

4. Section IV: Scenario Script Scenario Cast:

- **Patient:** Standardized Patient or Manikin (12-year-old with Pneumonia).
- **Confederates:** Actual KFSHRC staff performing assigned roles.

Required Systems & Equipment:

- **Siratech (HIS):** For registration, encounters, and clinical documentation.
- **Respiratory Equipment:** Nasal cannula (2L), O₂ flowmeter, and suction equipment.
- **Clinical Tools:** IV Pump, ID Armband printer, Vital signs monitor

Step/State	Patient Status/Action	Actions & System Triggers
1. Pre-Arrival	Pending transfer from external facility.	Duty Manager: - Performs registration - creates pre-arrival encounter in EMR. Take Inpatient Unit approval: 1. Bed assigned. 2. Admission request accepted. 3. Nurse assigned. 4. Room prepared. 5. O₂/Suction/Emergency equipment checked. 6. IV pump prepared. 7. Isolation setup ready (if required).
2. Arrival Phase (ER)	Arrives at EMS bay on 2L O ₂ .	ER Staff: Nurse performs triage; nurse notify the physician for patient review. Trigger: Physician completes assessment, documents transfer decision, and clears for admission. Duty Manager: Verifies ID, prints/applies armband, and completes inpatient encounter registration.
3. Transfer Phase	Moving from ER to B20 Floor.	EMS & Duty Manager: 1. Arrange transport. 2. Monitor patient equipment available and attached. 3. Send PT file/Medication chart. 4. Arrive at correct room.
4. Arrival to Room	Enters B20 Inpatient room.	Inpatient Nurse: 1. Receives patient. 2. Verifies 2 identifiers/ID band. 3. Safety check (ABC/General/ Bed low/rails up/call bell). 4. Vital signs. 6. Attach O₂ and IV infusions, If required 7. Handover (HX/Allergy/Interventions, medication). 8. Completes Nursing assessment in system 9. Notifies on-call MD.
5. Physician Arrival	MD enters room for admission.	Inpatient Physician: 1. Introduces self. 2. Reassessment. 3. Admission documentation. 4. Medication reconciliation. 5. Orders CBC (Lab readiness) 6. order Imodium (Pharmacy readiness). 7. Sample should be withdroan by nurses 8. medication and brough by phamracy team. 9. labs should be reported by lab.

5. Section V: Patient Data and Baseline State

Clinical Vignette: "The patient is a 12-year-old being transferred from an external hospital for management of Pneumonia. The patient requires IV antibiotics and continuous oxygen therapy via nasal cannula at 2 L/min." **Patient Profile:**

- **Diagnosis:** Pneumonia.
- **Therapy:** 2L O₂ via Nasal Cannula; IV Antibiotics.
- **Weight:** 50kg. **Baseline State:**
- **Vital Signs:** BP 120/80, HR 88, RR 20, O₂Sat 94% on 2L.

6. Section VII: Supporting Documents & System Verification

- **Laboratory Orders:** CBC ordered to test Lab readiness.
- **Pharmacy Orders:** Imodium ordered to test Pharmacy readiness and dispensing workflow.
- **Data Integrity:** Verify that the ID armband printed in the ER matches the Siratech master record for the inpatient encounter.

7. Section VIII: Debriefing Guide

Validation Questions:

- **Patient Safety:** Were two identifiers verified at the bedside before attaching O₂ and IV lines?
- **System Performance:** Did the "Pre-Arrival" encounter smoothly transition to an "Inpatient" encounter in Siratech?
- **Unit Readiness:** Were the O₂ and suction systems tested and functional *before* the patient reached the floor?
- **Inter-departmental Handover:** Was the medication chart and clinical history correctly communicated between the ER and B20 staff during the transfer phase?

Auditor checklist:

	steps	done	Not done	Partially done	comment
Prearrival	Registration process				
	Verify all 7 unit readiness steps are confirmed in the system.				
triage	Confirm ID verified/printed				
	Physician clearance documented in the note.				
Transfer	Validate patient monitoring				
	all required document available with the patient				
Room Arrival	Receives patient.				
	Verifies 2 identifiers/ID band.				
	Safety check (ABC/General/Bed low/rails up/call bell).				
	Vital signs				
	Attach O ₂ and IV infusions, If required				
	Handover (HX/Allergy/Interventions, medication).				
	Completes Nursing assessment in system				
	Notifies on-call MD.				
Physician	Validate medication reconciliation				
	order requests for medication and labs and time for each order to be completed and carried out				
	Admission note				